STATE OF WASHINGTON
DEPARTMENT OF HEALTH
PO Box 47874 • Olympia, Washington 98504-7874

September 29, 2020

Thomas Russell
Daybreak Youth Services - Brush Prairie
11910 NE 154th St
Brush Prairie, WA 98606-9571

Dear Mr./Ms. Thomas Russell:

This document contains information regarding the recent compliance monitoring survey of Daybreak Youth Services - Brush Prairie, 11910 NE 154th St, Brush Prairie, WA. 98606-9571 by the Washington State Department of Health. Your state licensing inspection was completed on 09/04/20.

During the inspection, deficient practice was found in the areas listed on the attached Statement of Deficiencies (Notice of Correction). A written Plan of Correction is required for each deficiency listed on the Statement of Deficiencies (Notice of Correction) and will be due 10 business days after you receive this document.

Each plan of correction statement must include the following:

- The regulation number;
- How the deficiency will be corrected;
- Who is responsible for making the correction;
- When the correction will be completed
- How you will assure that the deficiency has been successfully corrected. When monitoring activities are planned, objectives must be measurable and quantifiable. Please include information about the monitoring time frame and number of planned observations.

Please return the Plans of Correction to me at the following address:

Department of Health, Office of Health Systems Oversight
ATTN: RTF/BHA Investigation Unit
P.O. Box 47874
Olympia, WA 98504-7874

Enclosures: DOH Statement of Deficiencies (Notice of Correction)
Plan of Correction Brochure

Residential Treatment Facility
Inspection Report

Department of Health
P.O. Box 47874, Olympia, WA 98504-7874
TEL: 360-236-4732

September 24, 2020

Daybreak Youth Services - Brush Prairie, 11910 NE 154th St, Brush Prairie, WA. 98606-9571	Thomas Russell
Agency Name and Address	Administrator

COMPLIANCE WITH A DISCIPLINARY ORDER	08/10/20 – 09/04/20	Reviewer 33894, JAMC3, 30471
Inspection Type	Inspection Dates	Reviewer(s)
X2019-467	RTF.FS.60722961	Co-occurring Services, ,
Inspection Number	License Number	RTF Services Types

Please note that the deficiencies/violations/observations noted in this report are not all-inclusive, but rather were deficiencies/violations/observations that were observed or discovered during the inspection.

Deficiency Number and Rule Reference	Observation Findings	Plan of Correction
0555 Quality improvement program WAC 246-337-048(4) The licensee must establish policies and procedures to ensure ongoing maintenance of a coordinated quality improvement program to improve the quality of care provided to residents and to identify and prevent serious or unanticipated resident and facility outcomes. The licensee must:(4) Implement and document changes or improvements made to pre-vent future occurrences of any serious or unanticipated resident outcome specified in subsection (2) of this section.	Based on interview, email correspondence, facility document review and policy and procedure review, the facility failed to implement and document changes or improvements made to prevent future occurrences of any serious or unanticipated resident outcomes. Failure to implement and document changes or improvements made to prevent future occurrences of any serious or unanticipated resident outcomes can result in continued serious and unanticipated resident outcomes. Findings included: 1. Review of the facility's policy titled, "Quality Improvement Plan", Policy #702 revised 10/21/19, states, "Performance Improvement (PI), also called Quality Improvement (QI), can	

	be considered as a system that makes things better. It is a pro-active and continuous study of 'systems' that are causing poor outcomes. The intent of a PI program is to prevent or decrease the likelihood of problems by identifying areas of opportunity and testing new approaches to fix underlying causes of persistent systemic problems." 2. Review of the facility's monthly Quality Improvement Meeting Notes submitted to the department from December 2019 – August 2020 did not contain documentation that the committee reviewed serious or unanticipated resident or facility outcomes (i.e. incident reports), or documentation that any changes or improvements were implemented to prevent future occurrences of any serious or unanticipated resident outcomes. 3. During an interview on 09/23/20 at 2:23 PM, Staff B, Risk Management Assistant, stated that they are now starting to work on tracking incident trends. Incident reports are addressed in the Process Improvement Committee (PIC), which meets weekly. 4. Email correspondence received on 09/08/20 from Staff C, Director of Risk Management, contained PIC meeting notes for 03/19/20, 06/11/20, and 08/06/20. Review of the meeting notes showed that the facility is gathering information on serious or unanticipated resident outcomes (i.e. incident reports). The minutes did not contain the results or effectiveness of assigned follow up and action items. 5. During an interview on 09/08/20 at 1:30 PM, Staff F, Chief Executive Officer, stated that the facility has been gathering information to gain a base line, but have not started the process of assessing changes for effectiveness.	
0710 Infection control WAC 246-337-060(1)(a)(b) The licensee must implement and maintain an infection control program that prevents the transmission of infections and communicable disease among residents, staff, and visitors by:(1) Developing written policies and	Based on observation, interview, document review, and policy and procedure review the facility failed to ensure each resident's care is provided in an environment that prevents the transmission of infections and communicable disease among residents and staff by failing to perform hand hygiene, disinfect surfaces, and store medication in a clean manner.	

procedures for:(a) Hand hygiene; (b) Cleaning and disinfection.	Failure to ensure each resident's care is provided in an environment that prevents the transmission of infections and communicable disease among residents and staff risks resident and staff harm related to exposure to contaminated surfaces and communicable diseases. Findings included: 1. Review of facility policy titled, Policy: Infection Control", effective date 06/18/19, found it requires facility staff to follow standard precautions, including hand hygiene before and after each resident contact and after touching contaminated items. The policy included a list of infections, including influenza, that require precautions beyond the standard precautions identified in the policy. The list did not include COVID-19. 2. During observations on 08/20/20 beginning at 8:00 AM, the following was seen by Reviewers #1 and #3: a. Staff K, RN, had gloves on at the beginning of the observations. Staff K was not seen changing their gloves or performing hand hygiene between resident medication administrations or after touching contaminated surfaces, including the medication cart, the front of their facemask, a nasal spray and bottle of medication handled by residents, and vital sign equipment that touched residents' bare skin. b. A resident was observed self-administering a nasal spray handed to them by Staff K. After administering the spray into their nostrils, the resident handed the spray back to Staff K, who placed the inhaler inside a box in the medication cart. Staff K was not seen cleaning the sprayer prior to placing back into the cart drawer where medications for multiple residents were stored. c. A resident was observed self-administering a syringe of medication they drew from a bottle handed to them by Staff K. After administering the medication into their mouth, the	

	resident rinsed the syringe with water and handed the syringe and bottle to Staff K who placed the medication and syringe directly into the medication cart. Staff K was not seen cleaning the bottle or syringe prior to placing back into the cart drawer where medications for multiple residents were stored. d. Multiple residents were observed touching and leaning on the top of the medication cart. The surface was used for medication preparation and Staff K also rested their gloved hands on the cart top. No cleaning of the cart between residents who touched the surface was seen. 3. During an interview at the time of the observations, Staff K stated they wash their hands before med pass and if they become contaminated or touch residents' mouths during med pass. Staff K stated that they did not wash their hands during the observed med pass, except at the beginning and end of the activity.	
0925 Resident rights WAC 246-337-075(1)(a) The licensee must establish a process to ensure resident rights are protected in compliance with chapter 71.12 RCW, this chapter, and other applicable laws, and are based on the service types provided in the RTF. This process must address how the RTF will: (1) In an understandable manner, inform each resident or their personal representative, designee or parent or guardian, of the following:(a) All rights, treatment methods, and rules applicable to the proposed health care of the resident;	Based on facility document review and interview, the facility failed to inform each resident or their personal representative of all their rights applicable to the proposed health care of the resident for 10 of 10 residents reviewed (Residents #1 - #10). Failure to inform each resident or their personal representative of all their rights applicable to the proposed health care of the resident can result in the inability for a resident to make informed decisions about their care, which can result in resident harm and poor treatment outcomes. Findings included: 1. Review of the facility's policies and procedures showed that the facility has a policy titled, "Behavioral Health Services – Clinical, Individual Rights, WAC 246-341-0600". The policy does not address the resident rights applicable to the proposed health care of the resident listed in Residential	

	Treatment Facility (RTF) WAC 246-337-075. 2. During an interview on 08/25/20 at 9:00 AM with administrative staff that included Staff C, Director of Risk Management, Staff E, Clinical SUD Director, and Staff F, CEO, Reviewer #1 notified the staff that the reviewers were unable to locate the RTF resident rights in the policies and procedures received, and requested that the facility provide a copy of all of the rights that residents receive upon admission. Review of the Intake Packet documents the facility sent in response to the request did not contain the RTF resident rights applicable to the proposed health care of the resident listed in RTF WAC 246-337-075.	
0970 Resident rights WAC 246-337-075(2)(a)(g) The licensee must establish a process to ensure resident rights are protected in compliance with chapter 71.12 RCW, this chapter, and other applicable laws, and are based on the service types provided in the RTF. This process must address how the RTF will:(2) Treat each resident in a manner that respects individual identity, human dignity and fosters constructive self-esteem. Each resident has the right to:(a) Be free of abuse, including being deprived of food, clothes, or other basic necessities; (g) Have a safe and clean environment.	Based on record review, email correspondence and interview, the facility failed to establish a process that ensured the protection of each resident's right to be free of abuse and have a safe environment for 6 of 10 residents reviewed (Residents #1, #4, #6, #8, #9 and #10). Failure to establish a process that ensures the protection of each resident's right to be free of abuse and have a safe environment can result in resident harm, trauma, and poor treatment outcomes. References: 1. 246-337-050(2)(c)(e) Management of human resources. (2) A sufficient number of staff must be present on a twenty-four hour per day basis to: (c) Provide crisis intervention; and (e) Carry out required monitoring activities. 2. WAC 246-337-080 Resident care services. (2) The licensee must: (a) Limit admission, transfer, discharge, and referral processes to residents for whom the RTF is qualified by staff, services, equipment, building design and occupancy to give safe care. Findings included:	

1. Review of Resident #1's clinical record showed a physical altercation on 01/18/20 at 8:00 PM at a time of "low staff" that resulted in injuries as evidenced by the following:

a. Treatment Interfering Behavior(s) Report, dated 1/19/20, states, "[Resident #1] was sitting in the west side living room. When DM came out of his room [and] threw a table in his direction...turned in to fighting...[Resident #1] was bitten by DM and [Resident #1] then kicked his face and continued to punch until SC [Staff J, Skills Coach] had separated DM from the fight...due to low staff and not being able to conduct CPI [Crisis Prevention Institute] we did what we could with holing [sic] clients back and trying to separate them as best we could. No clients were letting go or slowing down the fight was hard to separate. [So] it was a lot of holding back and grabbing body's the best we could."

b. The section of the TIB report titled, "Staff in Area",' lists five staff in the area at the time of the incident.

c. Medical Feedback Note for Resident #1, dated 01/22/20, states, "Had his left ankle bit. Skin was removed...Also reports pain in the right hand."

2. Review of Resident #8's clinical record showed that on 01/21/20, Resident #8 was assaulted by another resident that resulted in an injury as evidenced by the following:

a. Treatment Interfering Behavior(s) Report, dated 01/21/20, states, "CL IJ was walking down the hallway while Staff TR and [Resident #8] were sitting in the living room talking...CL IJ...put his arm around [Resident #8's] neck and [Resident #8] turned around and started punching IJ in the face...threw punches back and forth until IJ kicked [Resident #8] in the face. Staff NB and staff SB interveined [sic] and broke up the fight."

b. The section of the TIB report titled, "How will the behavior(s) be addressed", states, "Client was transported to the ER for medical intervention. Client was informed he

needed 20 stitches though refused treatment as he does not like needles or doctors...[Resident #8] stated 'it is [f***ed] up that he kept kicking me when I was down'."

3. Review of Resident #10's clinical record showed that the resident was involved in bullying activities, including an assault on another resident as evidenced by the following:

a. ASAM review note, dated 02/26/20, states, "The client reports he is experiencing bullying from peers, the client himself is also engaging in bullying behaviors. Client was involved in an incident where he and some peers entered another [kid's] room; this client covered the camera while others beat up a kid who was sleeping."

4. Review of Resident #4's clinical record showed that on 03/09/20 at 9:00 AM, Resident #4 choked another resident while being unsupervised as evidenced by the following:

a. Treatment Interfering Behavior(s) Report (TIB), dated 03/10/20 states, "Client states that he choked client until his face turned red approx. 30 seconds...a few minutes later...he choked [client] for approx. 5 seconds the second time... Client reports that there was only one staff on the floor at the time and she was opening a bedroom door for one of the clients and did not see what had happened."

b. The section of the TIB report titled, "Staff in Area", lists one staff in the area at the time of the incident.

c. The section titled 'How will the behavior(s) be addressed' on the report states, "Client was placed on frozen for the week and a behavior contract will be drawn up at individual session." Review of the next Individual Session note, dated 03/12/20, did not address a behavior contract. No behavior contract was found in the resident's record.

5. Review of Resident #9's clinical record showed that on 03/12/20 the resident, who identifies as a cisgender male,

	was the victim of abuse that resulted in injuries, as evidenced by the following: a. Review of Treatment Interfering Behavior(s) Report, dated 03/12/20, states, "[Resident #9] came out of his room, with swollen eyes...immediately started crying. He said peer GM beat the backside of his head in the bathroom with his fists...As seen on Camera: [Resident #9] walked into bathroom alone, and went into back corner near the shower. Peer GM walked in alone, and went into the back corner where [Resident #9] had gone. (They both were out of sight for the camera.) Peers CS, TF, RW, walked in shortly after and went into the back corner, and can be seen moving around. All peers except [Resident #9] walked back into view of the cameras, in the E Hall Bathroom, and began rejoicing, jumping around and smiling. RW went into a stall briefly, then came out. [Resident #9] has come forward and can be seen being urged into a stall by peers. RO, TR, and KC also entered the bathroom, and were watching the aftermath." b. Review of Treatment Interfering Behavior(s) Report for resident GM, dated 03/12/20 9:39 PM, states, "[GM] was seen walking out of the shower...with peers laughing and high fiving each [other]...[Resident #9] walked out with a bump on the back of his head and blood in his mouth." c. Review of Medical Feedback note, dated 03/13/2020, states, "F/up with [Resident #9] after the incident happened last night...Palpated 4" diameter firm bump on his occipital region of his head, which has redness superficially...lower lip and L buccal have superficial laceration. [Resident #9] reported that he believes that he got hit by peer GM 30-40 [times] on the head...At 1700, [Resident #9] came back from ER...diagnosed with 'Closed head injury' and 'concussion without loss of consciousness'." d. Review of Treatment Interfering Behavior(s) Report, dated 03/12/20 for Resident #9, section titled "Description of Plan", states, "[Resident #9] was also placed on 24 hour watch".	

	Review of "Addenda", dated 03/19/20 at 10:35 AM, states, "Cl was put on 24's for the night". Review of Resident #9's clinical record showed no "Client 15 min or 24 HR Check Sheets" or other documentation showing that a 24 hour watch occurred. 6. Review of Resident #6's clinical record showed that the resident was at risk for suicide during their stay at the facility and attempted suicide on 06/20/20 as evidenced by the following: a. Review of Inpatient Discharge, dated 07/28/20, states that the reason the resident was at the facility was due to a recent DUI accident and that the resident was permitted by the Judge to attend inpatient treatment, "I got into a car accident with my buddy, he ended up dying. I was charged with vehicular man slaughter and went to jail...was there a week". b. Review of SUD Assessment, dated 05/28/20, shows that the resident had a history of 3 suicide attempts, a history of self-harm by cutting and burning himself, and that the resident stated, "When I was in jail I got pretty down. I got close to killing myself". c. Review of Treatment Interfering Behavior(s) Report, dated 06/20/20, states, "CL was found in the bathroom by himself by his peer CL EC with a blanket wrapped around his neck and the other end of blanket was tied on the restroom door in bathroom stall with stall door locked, CL EC said he had tried to untie him but was unable to and that is when CL EC ran to get staff...CL was transported to ER by paramedics". d. The record shows that the resident continued to have suicidal ideation when he returned to the facility. "Physician E&M Document", dated 06/25/20, states, "Continues with active plans – drown self in toilet, jump from bridge". Individual Session note, dated 06/26/20, states, "Client reported SI, specifying hanging or drowning self. Cl stated he does not want to make an unsuccessful attempt that does not end his life..." Individual Session, dated 07/10/20, states,	

	"CI reported worsening SI and identified specific plans..." e. Review of the facility's "Self-Harm/Suicidal Behavior Intervention Protocol", no date, states, "Monitor behavior as indicated on Self-Harm Assessment & Intervention Plan". Review of the resident's record showed no "Self-Harm Assessment and Intervention Plan" document. A Safety Plan was in the record, but this plan did not indicate how the resident was to be monitored. The resident was at the facility from 05/28/20 – 08/10/20. Inpatient Discharge, dated 07/28/20, states, "Throughout his stay in treatment, he remained on a 24 hour supervision to ensure safety." Review of the record showed that the record contained two "Client 15 min or 24 HR Check Sheets" for the 75 days the resident was at the facility, one for 06/19/20 and one with no date. The record contained 24 "Bed/Room Check Sheets" for the 75 days the resident was at the facility. The Bed/Room Check Sheets state that checks are to be done randomly every 20-30 minutes. Review of the sheets, dated 05/29/20 – 06/21/20, showed that 6 sheets documented 1 check for the day, 4 sheets documented 2 checks for the day, 6 sheets documented 3 checks for the day, 5 sheets documented 4 checks for the day, and the remainder of the sheets documented between 5 and 11 checks for each day. 7. Review of Resident #6's clinical record showed that on 08/03/20 at 8:30 AM, Resident #6 put another resident in a head lock which staff were unable to release as evidenced by the following: a. Treatment Interfering Behavior(s) Report, dated 08/03/20, states, "CL [client] ... put peer in a head lock. CL said '[he's] going to take a nap now' Staff tried to stop CL by pulling his hands off but CL was too strong for SC [Staff I, Skills Coach]." b. The section on the TIB report titled, "Staff in area", lists Staff I, Skills Coach, as the only staff in the area at the time of the incident. 8. Review of facility policies and procedures showed no	

policy or procedure regarding the use of cameras at the facility. During an interview on 08/25/20 at 9:00 AM with facility administrative staff that included Staff C, Director of Risk Management, Staff E, Clinical SUD Director, and Staff F, CEO, Reviewer #1 requested a copy of any policies or procedures the facility had addressing the use of video cameras at the facility. Email correspondence received on 08/27/20 from Staff C, Director of Risk Management, states, "After conducting our due diligence, DYS is unable to produce a Camera Use policy and procedure. We simply don't have one at this time."

9. During an interview on 08/14/20 at 11:00 AM with facility administrative staff that included Staff C, Director of Risk Management, Staff E, Clinical SUD Director, and Staff F, CEO, Reviewer #1 requested a copy of any policies or procedures the facility had addressing the monitoring of residents and special precautions, including those with self-harming behaviors. Email correspondence received on 08/18/20 from Staff C, Director of Risk Management, stated the following:

a. "Monitoring of clients/special precautions. Based on acuity and need, it is Daybreak's practice that direct patient care staff may implement 24 hour or 15-minute watches. Nursing, SUD, Mental Health personnel make a determination of whether to discontinue. *See Attachment 1*." Attachment 1 contained a copy of the "Client 15 min or 24 HR Check Sheet" and the "Bed/Room Check Sheet". No policy was attached.

b. "Self-harming protocols. It is Daybreak's practice that if DYS floor staff observe a patient is presenting with severe and/or acute emotional or cognitive symptoms or unmanageable behavior, staff will immediately intervene and notify patient's primary counselor. *See Attachment 2*." Attachment 2 contained a copy of the "Self-Harm/Suicidal Behavior Intervention Protocol" document and the "Self-Harm/Suicidal-Homicidal Assessment and Intervention Plan" document. No policy was attached.

10. During an interview on 09/04/20 at 10:05 AM, Staff R,

	Team Lead, stated that all residents at the facility are on the 15 minute bed/room checks. Review of "Bed/Room Check Sheets", dated 06/18/20 – 06/20/20, showed that the sheets document room numbers and not resident names. Some of the sheets documented only 2 checks for the entire day. Most of the sheets did not document bed/room checks during the night hours when residents should have been in their beds sleeping.	
1035 Resident rights WAC 246-337-075(6) The licensee must establish a process to ensure resident rights are protected in compliance with chapter 71.12 RCW, this chapter, and other applicable laws, and are based on the service types provided in the RTF. This process must address how the RTF will:(6) Account for each resident's assets, including allowance, earnings from federal or state sources and expenditures.	Based on record review and interview, the facility failed to ensure the protection of each resident's right to have their assets accounted for by the facility for 8 of 10 residents reviewed (Residents #1, #2, #3, #4, #5, #7, #8, and #10). Failure to protect each resident's right to have their assets accounted for by the facility can result in the loss of resident property. Findings included: 1. Review of clinical records for Residents #1, #2, #3, #4, #5, #7, #8 and #10 showed that the records did not contain an inventory list of the resident's belongings. 2. An email received on 08/12/20 from Staff B, Risk Management Assistant, in response to a request for documentation accounting for each resident's belongings, Staff B wrote, "Due to not having a strong process in place I am unable to find the other 8 clients inventory." 3. Review of the inventory lists for Resident #6 and #9 showed that they account for each resident's belongings on admission, but do not account for each resident's belongings on discharge.	

Introduction
We require that you submit a plan of correction for each deficiency listed on the inspection report form. Your plan of correction must be submitted to the DOH within ten business days of receipt of the list of deficiencies.

You are required to respond to the Inspection Report with Noted Deficiencies by submitting a plan of correction (POC). Be sure to refer to the deficiency number. If you include exhibits, identify them and refer to them as such in your POC.

Descriptive Content
Your plan of correction must provide a step-by-step description of the methods to correct each deficient practice to prevent recurrence and provide information that ensures the intent of the regulation is met.

An acceptable plan of correction must contain the following elements:

- The plan of correcting the specific deficiency;
- The procedure for implementing the acceptable plan of correction for the specific deficiency cited;
- The monitoring procedure to ensure that the plan of correction is effective and that specific deficiency cited remains corrected and/or in compliance with the regulatory requirements;
- The title of the person responsible for implementing the acceptable plan of correction.

Simply stating that a deficiency has been "corrected" is not acceptable. If a deficiency has already been corrected, the plan of correction must include the following:

- How the deficiency was corrected,
- The completion date (date the correction was accomplished),
- How the plan of correction will prevent possible recurrence of the deficiency.

Completion Dates
The POC must include a completion date that is realistic and coinciding with the amount of time your facility will need to correct the deficiency. Direct care issues must be corrected immediately and monitored appropriately. Some deficiencies may require a staged plan to accomplish total correction. Deficiencies that require bids, remodeling, replacement of equipment, etc., may need more time to accomplish correction; the target completion date, however, should be within a reasonable and mutually agreeable time-frame.

Continued Monitoring
Each plan of correction must indicate the appropriate person, either by position or title, who will be responsible for monitoring the correction of the deficiency to prevent recurrence.

Checklist:

- Before submitting your plan of correction, please use the checklist below to prevent delays.
- Have you provided a plan of correction for each deficiency listed?
- Does each plan of correction show a completion date of when the deficiency will be corrected?
- Is each plan descriptive as to how the correction will be accomplished?
- Have you indicated what staff position will monitor the correction of each deficiency?
- If you included any attachments, have they been identified with the corresponding deficiency number or identified with the page number to which they are associated?

Your plan of correction will be returned to you for proper completion if not filled out according to these guidelines.

Note: Failure to submit an acceptable plan of correction may result in enforcement action.

Approval of POC

Your submitted POC will be reviewed for adequacy by DOH. If your POC does not adequately address the deficiencies in your inspection report you will be sent a letter detailing why your POC was not accepted.

Questions?

Please review the cited regulation first. If you need clarification, or have questions about deficiencies you must contact the inspector who conducted the onsite inspection, or you may contact the supervisor.

NOV 0 2 2020

Office of Health Systems Oversight

DAYBREAK youth services

Plan of Correction

RTF.FS.60722961

Deficiency Number	Rule Reference	Observation of Findings	How the deficiency will be corrected?	Who is responsible for making the correction?	When correction will be completed?	Monitoring
0555 Quality improvement program	WAC 246-337-048(4) The licensee must establish policies and procedures to ensure ongoing maintenance of a coordinated quality improvement program to improve the quality of care provided to residents and to identify and prevent serious or unanticipated resident and facility outcomes. The licensee must:(4) Implement and document changes or improvements made to prevent future occurrences of any serious or unanticipated resident outcome specified in subsection (2) of this section.	Based on interview, email correspondence, facility document review and policy and procedure review, the facility failed to implement, and document changes or improvements made to prevent future occurrences of any serious or unanticipated resident outcomes. Email correspondence received on 09/08/20 from Staff C, Director of Risk Management, contained PIC meeting notes for 03/19/20, 06/11/20, and 08/06/20.	Daybreak implemented new Quality Improvement and process improvement policies and procedures. It has identified quality metrics and has established baseline data which will be used to document effectiveness of quality improvement initiatives. Daybreak's Process Improvement Committee will implement a standing agenda item, "Follow Up for Effectiveness",	Michael Trotter, Risk Management Director with support of Melissa Boyd, Risk Management Assistant.	12/31/2020	Daybreak's Risk Management Department will review PIC Meeting Minutes to ensure that documented changes and goals are implemented.

		Review of the meeting notes showed that the facility is gathering information on serious or unanticipated resident outcomes (i.e. incident reports). The minutes did not contain the results or effectiveness of assigned follow up and action items	to ensure documented changes, plans, and goals are implemented and tracked to demonstrate effectiveness. The base line and subsequent data will be used to support this work. Effectiveness is evaluated at 30, 60, and 90 days to ensure documented changes, plans, and goals are met.			
Deficiency Number	**Rule Reference**	**Observation of Findings**	**How the deficiency will be corrected?**	**Who is responsible for making the correction?**	**When correction will be completed?**	**Monitoring**
0710 Infection Control	WAC 246-337-060(1)(a)(b) The licensee must implement and maintain an infection control program that prevents the transmission of infections and communicable disease among residents, staff, and visitors by:(1) Developing written policies and procedures for:(a) Hand hygiene;(b) Cleaning and disinfection.	Review of facility policy titled, Policy: Infection Control" the list did not include COVID-19. During observations on 08/20/20 beginning at 8:00 AM, the following was seen by Reviewers #1 and #3: Staff K was not seen changing their gloves or performing hand hygiene between resident medication	The Infection Control Policy has been sent to the Medical Director for revision to include COVID-19 related procedures. To address the immediate need, training was completed on October 13, 2020.	Dr. Keith Johnson, Medical Director. Becky Pulito, Director of Nursing.	Infection Control Policy revision will be complete by 11/1/2020. October 13, 2020	The Infection Control Policy has been sent to the Medical Director for revision and approval to include COVID-19 and procedures that address appropriate hand hygiene and cleaning of the medication cart and other patient

		administrations or after touching contaminated surfaces A resident was observed self-administering a nasal spray handed to them by Staff K. After administering the spray into their nostrils, the resident handed the spray back to Staff K,who placed the inhaler inside a box in the medication cart. After administering the medication into their mouth, the resident rinsed the syringe with water and handed the syringe and bottle to Staff K, who placed the medication and syringe directly into the medication cart. No cleaning of the cart between residents who touched the surface was seen. Staff K stated that they did not wash their hands during the observed med pass, except at the beginning and end of the activity.	The nurses were trained on changes in procedure and practice regarding proper hand hygiene to ensure the prevention of the transition of infections and communicable disease.			care equipment to ensure the prevention of the transmission of infectious and communicable disease. To address the immediate need, training was completed on October 13, 2020. The nurses were trained on changes in procedure and practice regarding proper hand hygiene as well as the cleaning of the med cart and other client care equipment to ensure the prevention of the transition of infections and communicable disease

Deficiency Number	Rule Reference	Observation of Findings	How the deficiency will be corrected?	Who is responsible for making the correction?	When correction will be completed?	Monitoring
0925 Resident rights	WAC 246-337-075(1)(a) The licensee must establish a process to ensure resident rights are protected in compliance with chapter 71.12 RCW, this chapter, and other applicable laws, and are based on the service types provided in the RTF. This process must address how the RTF will: (1) In an understandable manner, inform each resident or their personal representative, designee or parent or guardian, of the following:(a) All rights, treatment methods, and rules applicable to the proposed health care of the resident.	Review of the facility's policies and procedures showed that the facility has a policy titled, "Behavioral Health Services –Clinical, Individual Rights, WAC 246-341-0600". The policy does not address the resident rights applicable to the proposed health care of the resident listed in Residential Treatment Facility (RTF) WAC 246-337-075.2 Review of the Intake Packet documents the facility sent in response to the request did not contain the RTF resident rights applicable to the proposed health care of the resident listed in RTF WAC 246-337-075.	The deficiency was corrected by adding the Resident Rights – WAC 246-337-075 to the Parent Guardian Packets given to clients at intake to inform them of their individual rights.	Michael Trotter, Director of Risk Management	10/15/2020	The Director of Risk Management or his designee, will observe the admission process 5 times during the first two weeks of implementation. Thereafter Risk Management staff will observe one admission per week for 30 days. The Director of Risk Management, or his designee, will audit all client files for the first 30 days after implementation to assure that each client has attested in writing to receipt of the resident rights. See Attachment 1 for Resident Rights

Deficiency Number	Rule Reference	Observation of Findings	How the deficiency will be corrected?	Who is responsible for making the correction?	When correction will be completed?	Monitoring
0970 Resident Rights	WAC 246-337-075(2)(a)(g) The licensee must establish a process to ensure resident rights are protected in compliance with chapter 71.12 RCW, this chapter, and other applicable laws, and are based on the service types provided in the RTF. This process must address how the RTF will:(2) Treat each resident in a manner that respects individual identity, human dignity and fosters constructive self-esteem. Each resident has the right to:(a) Be free of abuse, including being deprived of food, clothes, or other basic necessities;(g) Have a safe and clean environment.	Based on record review, email correspondence and interview, the facility failed to establish a process that ensured the protection of each resident's right to be free of abuse and have a safe environment for 6 of 10 residents reviewed (Residents #1, #4, #6, #8, #9 and #10). See SOD for more detail	Daybreak will develop a Resident Supervision policy describing Daybreak's overall approach to resident supervision. It will incorporate staff training, protocols for supervision, suicide prevention, incident intervention, forms and tools designed to support and guide staff in their supervision of clients, and other topics related to creating and maintaining a safe environment for the clients.	Becky Pulito, Director of Nursing with the support of Angela Ball, SUD Clinical Director and Sandra Skok, MH Clinical Director	The policy will be complete by 11/30/2020 Staff will be trained by 12/31/2020 The policy and related procedures will be fully implemented by 1/31/2021	Daybreak's Director of Nursing will review 4 randomly selected client files bi-weekly for three (3) months and Monthly thereafter to ensure compliance with the Resident Supervision Policy and related protocols to verify implementation.

Deficiency Number	Rule Reference	Observation of Findings	How the deficiency will be corrected?	Who is responsible for making the correction?	When correction will be completed?	Monitoring
1035 – Resident Rights -	WAC 246-337-075(6) The licensee must establish a process to ensure resident rights are protected in	Based on record review and interview, the facility failed to ensure the protection of each	Daybreak revised the Inventory Checklist by adding a signed acknowledgement by the	Melissa Boyd, Risk Management Assistant.	11/1/2020	Risk Management staff will review every inventory

	compliance with chapter 71.12 RCW, this chapter, and other applicable laws, and are based on the service types provided in the RTF. This process must address how the RTF will:(6) Account for each resident's assets, including allowance, earnings from federal or state sources and expenditures.	resident's right to have their assets accounted for by the facility for 8 of 10 residents reviewed	resident that their property was accounted for at intake and returned at discharge to ensure the protection of each resident's right to have their assets accounted for by the facility.			sheet at the time of admission and at the time of discharge for from 11/1/2020 through 11/30/2020 to assure implementation.